

Co-Designing Reproductive Justice Futures: Intersectional Care-Activism for Abortion Access

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ABSTRACT

This dissertation project will integrate Feminist HCI and Reproductive Justice to interrogate how current technologies being used to address gaps in abortion access risk perpetuating reproductive oppression, and co-design supports for inclusive abortion access. My research looks at how the current "crisis of care" in abortion access resulting from the 2022 *Dobbs* decision is responsabilizing technologies and putting unsustainable demand on Abortion Access Organizations while not addressing underlying systemic barriers to abortion access which were in effect prior to 2022. My project focuses on Illinois, USA, where many abortion seekers are traveling to access their care. I use qualitative methods and design as research to explore how AAOs are currently using technologies, how extant abortion technologies are leaving out particular user groups, and how design can work towards Reproductive Justice futures.

CCS CONCEPTS

• **Human-centered computing** → **Collaborative and social computing theory, concepts and paradigms; Collaborative and social computing systems and tools**; • **Social and professional topics** → **Women**.

KEYWORDS

abortion, care, activism, reproductive justice, design

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1 INTRODUCTION

My dissertation will integrate feminist HCI design principles and Reproductive Justice (RJ) to support care work in abortion access in the state of Illinois, USA to address the intersectional needs of people seeking abortions. I will investigate the way Abortion Access Organizations (AAOs) and abortion seekers are leveraging technologies to fill in gaps in abortion access, and integrate community-oriented approaches to abortion activism in envisioning a future approach to technology's role in abortion.

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My research employs care as a foundational theory, drawing on the current "crisis of care" narrative used to describe the state of abortion in the United States. I will build on existing theories of care in HCI that advocate for care-based change and introduce the work of abortion access and reproductive justice organizers as a novel theory for HCI: *care-activism*. This theory emphasizes care as a catalyst for change and a means to shape future worlds, rather than solely focusing on maintenance. It integrates intersectional theories of care from BIPOC and trans theorists, highlighting the transformative potential of care in driving social justice and innovation. My research will go through two periods of field work and partner with AAOs to co-design for RJ futures. The dissertation will conclude with the creation of a resource for use by abortion activists, and potentially the public developed in partnership with abortion access activists.

2 CONTEXT AND MOTIVATION

Access to abortion is a human right. Nearly two years ago, the Supreme Court of the United States' *Dobbs* decision revoked the national legal protection for abortion that had been in place for nearly 50 years, resulting in what has been termed a "crisis of care". Illinois in particular has become a center of abortion access, due to its abortion-protective laws and proximity to abortion-restrictive states, with hundreds of abortion seekers traveling to the state each week [10, 24]. As many people must now travel across state borders for care, requests for funds and support have skyrocketed [11]. While AAOs are a vital support for people seeking abortion care, they cannot keep up with growing demand and constricting access to abortion care. While the *Dobbs* decision was indeed a pivotal moment in abortion care in the United States, access to abortion has always been unevenly accessible across marginalized groups [23]. This begs the question, for whom is this a crisis, and who is being excluded by current efforts to address it?

2.1 Technologies for Abortion Access

In response to the heightened barriers and restrictions on abortion access, new technologies are being developed and existing technologies are being repurposed to support abortion access. For example, mobile applications have been created to walk someone through the self-managed medication abortion process [26], chatbots are now included on some abortion activism sites to help people navigate finding an abortion in the increasingly complex legal landscape [12], and groups on social media spaces have formed to answer any concerns around abortion access [21]. In the "care crisis," where dwindling resources for abortion are exacerbating gaps, care is being delegated to technologies. However, without the intentional inclusion of marginalized identities in the design process, these

technologies present a narrow image of who gets abortions and who needs attention in this "crisis," attempting to approximate the pre-*Dobbs* system, which never served everyone.

While digital technologies for abortion access are addressing critical gaps in care, they often reflect a narrow perspective on who is seeking abortions and can perpetuate systemic exclusions in reproductive health. With abortion laws changing rapidly and technologies stepping in to bridge these gaps, it is essential to intentionally design these technologies in collaboration with community members already engaged in abortion care work. This approach should aim to move beyond the current "crisis" and towards more inclusive futures for abortion care.

3 RELATED WORK

Research in HCI has already looked into digital technologies for abortion activism in protesting [15] and abortion story sharing [16]. However, thus far there has not been any work done on the role of technologies in direct provision of abortion care in the HCI space. Research in HCI is increasingly interested in reproductive health technologies for "intimate care" [1] such as menstrual care [5], polycystic ovarian syndrome [6], and breastfeeding [3]. Notably, while HCI research around reproductive health often leverages theories of care, research on abortion focuses only on political activism, signaling the politicization of abortion care.

Technologies around communicating for community-managed abortions have already been studied in the STS space, primarily in relation to feminist self-help groups [17]. Information around abortion and other issues have been shared through simple communication technologies, such as the book *Our Bodies, Ourselves* [18] and zine-like publications. However, historical technologies for abortion have been criticized for privileging white, middle-class, cisgender, non-disabled women [17].

4 THEORETICAL APPROACH

4.1 Care

In my dissertation, I will focus on care through a feminist HCI framework, to challenge the "crisis of care" framing, and to contribute to the way care is currently theorized in HCI. Currently, care in HCI focuses on care as a maintenance activity [27]. However, care theorists from marginalized groups—who are the most marginalized by current abortion policies—such as trans, Black, and disabled theories have not been centered in HCI, and offer new perspectives of care such as interdependency [20], mutual aid [25], and women-centered networks [7].

By taking a feminist theoretical framing, I hold each of these to be a valid understanding of care, and will work to incorporate these multiple care-worlds into my understanding of care, both independently and intersectionally. In asking how care can change and disrupt, in addition to maintain, I connect the effects of care to those of activism, where caring for a 'world' that has not yet come into being is a form of both care *and* activism. The RJ movement, for example, is an expression of care-activism, as organizers aim to address societal oppression through care-based action.

4.2 Reproductive Justice

The *Reproductive Justice* (RJ) movement was formed by a group of Black feminists during the period when abortion was nationally legalized, to highlight how access was not equally available across the country [23]. RJ frames reproductive agency as a human right, and calls attention to the way that women of color, trans men, people with disabilities, and people with financial barriers have always been prevented from choosing whether or not to have a child. RJ call for systemic change, drawing on historical evidence of reproductive injustice and contemporary oppression. I will be leveraging RJ to shape my intersectional understanding of reproductive oppression and how to mindfully address systemic barriers to reproductive autonomy in my research. Furthermore, I will select AAOs to work with who are practicing with RJ values, and already explicitly addressing marginalization in the abortion space.

4.3 Feminist HCI

I will use Feminist HCI [4] as my methodology. The key values drawn from feminist theories into feminist HCI align closely to key values from care theories, and calls to incorporate more theories of care into HCI research have emerged from feminist researchers [8]. These include the value-full nature of research where the attention of research brings realities into being by the researcher caring for that research [14]; the value of standpoint in reality and the interconnectedness of humans, technologies, and their environments [9]; and the imperative to attend to issues of gender [13].

5 RESEARCH QUESTIONS

My research aims to answer the following research questions:

- (1) How might HCI research document and support current efforts for abortion access?
- (2) How might technologies be able to facilitate sustainable, community-centric, RJ oriented futures for reproductive care?
- (3) What might an intersectional, activist theorization of care for HCI look like?

6 METHODS

To answer these questions I plan two periods of field work and a co-design stage, as well as a critical review of extant abortion technologies. During the field work phases, I will travel to Illinois, observe AAOs places of practice, and conduct interviews with AAO workers about their work, the challenges they face, and how technologies currently play a role in their work. I am already actively volunteering with some of these organizations online, and spent the last year forming relationships with a range of clinics, abortion funds, practical support networks, and political activists. The first phase will take place in July 2024, and the second will take place in January 2025.

For the critique of extant abortion technologies, I will be conducting a walkthrough review [22] of existing abortion applications, taking a critical lens in partnership with an intersectional group of RJ in HCI researchers. This work is already underway, with expected results to be submitted for publication in Fall 2024.

Next, I will conduct co-design workshops with AAO workers. I will be using workshop designs from the Community Led Co Design toolkit [2] and my own designs, based on the input of AAOs

from the interview sessions. Therefore, although I do not currently know what the co-design workshops will consist of pending empirical results from interviews, I plan on using extant successful co-design structures as scaffolding, which have been shown to fulfill justice and equity principles for design [19]. Based on the current challenges faced by AAOs as well as existing inequities in abortion access, I anticipate outcomes from the workshops will support existing efforts of AAOs, but also constitute novel approaches to reproductive care rooted in RJ values.

7 CONTRIBUTIONS

7.1 Contributions to date and Works in Progress

Thus far I have conducted research focusing on issues of justice and care in HCI, including a critique of equity design toolkits, as well as research on community-oriented care work. I have co-authored research on activism movements, and hosted workshops on feminist HCI and design for activism.

I have several research projects in progress, with impending publication outcomes. I have completed a literature review of care papers in HCI, with anticipated submission for publication in summer 2024. As mentioned, I am currently working alongside a group of transnational and intersectional RJ in HCI researchers to review abortion-related mobile applications, which we will be writing for fall 2024 publication deadlines.

7.2 Future Contributions

Resulting from my future work, I aim to have a collection of research papers, a public scholarship project, and a usable design outcome. Resulting from my interview field work, I aim to publish an account of the AAOs' use of technologies and how it has evolved after the *Dobbs* decision. I also aim to publish an inclusive design-oriented critique of emergent abortion technologies. Finally, I aim to publish the results of my co-design work, presenting a RJ oriented design and account of a design method that negotiates care, activism, intersectional oppression, and legal and institutional hostilities.

While pursuing academic publications, I will also be conducting a public scholarship project: a podcast to record and publish stories of AAO workers. This project is part of a PhD fellowship for public impact, and will be published for the general public over the 2024/2025 academic year.

Interwoven in my projects, I aim to shift narratives of abortion in the HCI space to integrate care with activism. Emergent from this goal, I will present a novel theorization of care-activism for HCI which interweaves care and activism to orient future scholarship towards care as a world-changing and world-building tool.

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